Op. Dr. Murat Ayata
Kulak Burun Boğaz
Otolaryngology Head and Neck Surgery Specialist

Op. Dr. Murat Ayata Private Clinic
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INFORMED CONSENT FORM

Rhinoplasty

Cosmetic nose surgery (combined with correction of the nasal airway if needed)

Dear patient, this form has been prepared to explain, in **plain and clear language**, what you need to know about the procedure recommended to you. Its purpose is not to frighten you, but to help you make your decision **freely and on the basis of good information**.

Please read this form without rushing. Ask your doctor about anything you do not understand. Your doctor will also explain verbally any risks that are particular to your situation. Whether or not you give consent is entirely your right.

PATIENT AND PROCEDURE DETAILS

Full name: _____ Date of birth: _____

Passport / ID No: _____ File / Protocol No: _____

Telephone: _____ Date informed: _____

Planned procedure date: _____

Form first sent to patient (date / e-mail, WhatsApp, in person): _____

Doctor performing the procedure and providing information: **Op. Dr. Murat Ayata**

Hospital / facility where the procedure will be performed: _____

Planned technique: ☐ Open technique ☐ Closed technique ☐ To be decided by the surgeon during the operation

Procedures to be performed at the same time: ☐ Septoplasty ☐ Turbinate reduction ☐ None _____

Source of cartilage graft: ☐ Inside the nose (septum) ☐ Ear ☐ Rib ☐ Not expected to be needed _____

Patient's concerns and expectations: _____

1 Your condition and the aim of the procedure

Rhinoplasty is an operation to reshape the outer appearance of the nose (the bridge, a hump, the tip, the width of the nose, the nostrils and its harmony with the face). If you have a bend in the central wall of the nose that makes breathing difficult (septal deviation), collapse of the nasal sidewalls or enlarged turbinates, these can be corrected in the same operation.

The shape of the nose results from inherited features, changes during growth or previous injuries (a blow, a fracture). These structures do not change by themselves. The aim of the operation is to reach the **realistic goals** we agree on together and to achieve a nose that is **natural, in harmony with your face and comfortable to breathe through**.

2 How is the procedure performed?

The operation is performed under general anaesthesia in an operating theatre. Before surgery your nose is examined and photographed, and a plan is made together with you.

1. **Incision:** In the closed technique all incisions are inside the nose and no scar is visible from outside. In the open technique a small incision of about 4–5 mm is also made across the thin bridge of skin between the nostrils (the columella); this scar usually becomes barely noticeable over time.

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2. **Reshaping the nasal framework:** The skin and soft tissues are gently lifted. If there is a hump, it is reduced from the bone and cartilage or corrected with hump-preserving techniques. The tip cartilages are reshaped with stitches.
3. **Narrowing/straightening the bones (osteotomy):** If wide or crooked nasal bones need to be narrowed or straightened, controlled cuts are made in the bone. This step may increase the chance of bruising and swelling.
4. **Support and grafts:** To help the nose keep its shape in the long term and keep the airway open, cartilage supports (grafts) are used if needed. Cartilage is taken first from inside the nose (the septum); if this is not enough, ear or rib cartilage may be used. This will be discussed with you before the operation.
5. **Functional correction:** If necessary, the central wall of the nose (septum) is straightened and the turbinates are reduced.
6. **Closure and splint:** Incisions are closed with fine stitches. A protective splint is placed over the bridge of the nose; if needed, soft silicone splints or packs that allow you to breathe are placed inside the nose.

ANAESTHESIA	APPROXIMATE DURATION	HOSPITAL STAY / DISCHARGE
General anaesthesia	About 2–4 hours	Usually one night in hospital

3 Expected benefits and likelihood of success

- A nose shape that is more natural, balanced and in harmony with your face
- Relief of nasal blockage, if present, and easier breathing
- Helping you to feel better about yourself

Most patients are satisfied with the result. However, the nose sits in the centre of the face and takes a long time to heal; the final result becomes apparent within **12–18 months**. Published studies report that about **5–15%** of patients may need a minor or more extensive corrective operation (revision). Results are harder to predict in noses with thick skin, previous surgery or a history of severe injury.

4 Other treatment options

- **No surgery:** As this is a cosmetic procedure, you always have the option not to have it.
- **Shaping the nose with fillers ("non-surgical rhinoplasty"):** Can temporarily camouflage a small hump or dip; it does not make the nose smaller, does not improve breathing and carries its own risks, such as blockage of a blood vessel.
- **Functional surgery only (septoplasty/turbinate reduction):** If breathing is the only problem, this can be done without changing the outer shape of the nose.
- **Medical treatment:** Nasal sprays may help with blockage caused by allergy or swelling of the lining; they do not correct structural deformities.

5 What happens if the procedure is not performed?

If the operation is not performed, the current shape of your nose will remain unchanged. If you have breathing problems with a structural cause (a deviation, sidewall collapse), these will not improve by themselves, but they usually do not pose an urgent danger to your health.

You have the right to decline the recommended procedure, to postpone it, or to seek a second medical opinion. This decision will not affect any other healthcare offered to you in any way. You may withdraw your consent at any time before the procedure begins; please be aware, however, that once a procedure has started there may be stages at which stopping would be medically unsafe.

6 Preparing for the procedure

- If you have taken **isotretinoin** (acne medicine, e.g. Roaccutane) in the last 6–12 months, please tell us; it can affect skin healing and the timing of surgery may be planned accordingly.

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- Blood tests and, if necessary, other investigations are carried out before surgery, and you will be assessed by the anaesthetist.
- You must not eat or drink anything for **6–8 hours** before the operation (please follow the anaesthetist's instructions).
- On the day of surgery do not wear make-up, nail polish, jewellery or contact lenses. It is advisable to have a relative or friend with you for the first day after surgery.
- Surgery may be postponed if you have active acne or a skin infection on the face, flu or an upper respiratory tract infection.
- Tell your doctor about **all medicines** you take (including blood thinners, aspirin, painkillers, weight-loss and hormone medicines) and any **herbal products and supplements** (such as omega-3, vitamin E, ginkgo, garlic, ginseng, fish oil). Do not stop any medicine on your own without your doctor's advice.
- Be sure to tell us about any **allergies**, bleeding or clotting problems, heart, lung, diabetes, blood pressure or thyroid conditions, previous operations and any problems you have had with anaesthesia.
- If there is any chance you may be pregnant, or if you are breastfeeding, tell us before the procedure.
- Smoking and nicotine products significantly impair wound healing. If possible, stop at least 2–4 weeks before the procedure and do not smoke during recovery.

7 Recovery: what to expect

First 48 hours	Nasal blockage, mild ooze, headache and swelling of the face are normal. Pain is usually mild to moderate and controlled with painkillers. Sleeping with your head raised and using cold compresses help to reduce swelling. **If** bruising occurs, it appears around the eyes and fades in 7–10 days.
Week 1	Any silicone splints/packs inside the nose are usually removed on days 2–5, and the external splint and stitches at around **day 7**. You will be asked to rinse the inside of the nose with saline. Most patients return to desk work and social life within 7–10 days.
Weeks 2–6	Most of the swelling settles, but the tip may still feel firm and mildly numb. Strenuous exercise, contact sports and swimming should be postponed for about **4–6 weeks**. Glasses should not be worn for at least **6 weeks** unless they are taped so that they do not press on the bridge of the nose.
Months 3–12	The remaining swelling, particularly at the tip, gradually decreases. The shape of the nose continues to change during this period; minor irregularities seen early on usually disappear.
Months 12–18	The final result becomes apparent. If a corrective (revision) procedure is considered necessary, it is usually done after waiting at least 12 months.

8 Possible risks and complications

As with any medical procedure, this procedure carries some risks. Below, **expected temporary effects** and **complications** are listed separately and grouped by how often they occur. The figures given are **approximate** values based on the medical literature; your personal risk may be lower or higher depending on your age, general health, smoking and previous operations.

The great majority of complications are **temporary** and improve with appropriate treatment. Very rare but serious events are also included; being on this list does not mean they will happen to you, but you need to know about them to make an informed decision.

Expected, temporary effects (not complications — a normal part of healing)

- Nasal blockage, mild ooze and blood-stained discharge (in the first few days)
- Swelling of the face and around the eyes; **if** bruising occurs, it fades within 1–2 weeks

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- Temporary numbness of the nasal tip and front teeth; firmness of the nasal tip
- Temporary reduction in sense of smell (improves as swelling settles)
- The shape of the nose continuing to change over months

Common conditions (most are temporary or resolve with treatment)

- **Prolonged swelling** — Especially with thick skin, swelling of the tip may last a year or longer. It can be helped with steroid injections if necessary.
- **Mild asymmetry and small irregularities** — Because the face itself is not perfectly symmetrical, slight differences may remain in the nose; small bony/cartilage bumps may be felt but are usually not visible.
- **Need for revision (correction)** — In about **5–15 out of 100** patients, a corrective procedure ranging from a minor touch-up to a more extensive operation may be needed.
- **Crusting and dryness inside the nose** — Common in the first few weeks; settles with regular rinsing and moisturising.

Uncommon conditions

- **Nosebleed** — In about **1–3 out of 100** patients, bleeding requiring packing or intervention may occur. Very rarely, the bleeding may need to be stopped in the operating theatre.
- **Infection** — Occurs in **anywhere from fewer than 1 to 3 in every 100** patients. It usually settles with antibiotics; rarely, the infection may need to be drained or a graft removed.
- **Breathing does not improve as expected** — Your nasal blockage may not fully resolve or, rarely, may increase; further treatment or surgery may be needed.
- **Hole in the septum (perforation)** — A hole in the central wall of the nose occurs in about **1 in 100** patients. It often causes no symptoms; if it causes whistling, crusting or bleeding, treatment may be needed.
- **Collection of blood or pus in the septum (haematoma / abscess)** — When the septum is operated on, this occurs in **fewer than 1 in 100** patients. It shows as increasing blockage, pain and fever in the days after surgery. It must be **drained urgently** to avoid damage to the nasal cartilage; if untreated, loss of cartilage and collapse of the nasal bridge (saddle nose) may occur, and further surgery may be needed to correct it.
- **Adhesions inside the nose (synechiae)** — Tissues inside the nose sticking together; if they cause blockage they can be separated with a simple procedure.
- **Noticeable scar** — In the open technique the columellar scar may rarely remain noticeable; if the nostrils are narrowed, a fine scar forms in the crease of the nostril.
- **Prolonged numbness** — Reduced sensation of the nasal tip or front teeth may last several months; rarely it may be permanent.
- **Graft-related problems** — Cartilage grafts may shift, bend, partially absorb or become visible through the skin over time. If cartilage is taken from the ear or rib, there is a risk of pain, scarring and bleeding at that site and, for the rib, very rarely injury to the lining of the lung (pneumothorax).

Rare or very rare, but potentially serious conditions

- **Skin problems** — Thinning of the skin, redness, small visible blood vessels, or increased scar tissue in thick skin. Very rarely, especially in smokers and noses with previous surgery, poor blood supply to the skin may lead to a wound and tissue loss. In that case prolonged wound care and reconstructive surgery may be needed, and a permanent scar or deformity may remain.
- **Permanent reduction or loss of sense of smell** — Very rare.
- **Tear-duct problems and bleeding around the eye** — Very rarely, the bone cuts may lead to a watery eye or bleeding around the eye.

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- **Cerebrospinal fluid (CSF) leak, meningitis, loss of vision** — Extremely rare complications, occurring in **far fewer than 1 in 10,000** cases, caused by injury to the thin bones that separate the nose from the skull base and the eye. If they occur, further hospital treatment (sometimes in intensive care) and surgery may be needed; loss of vision, if it occurs, may be permanent.
- **Toxic shock syndrome** — A very rare but serious illness linked to the use of nasal packs (sudden high fever, rash, low blood pressure) that needs urgent treatment. It requires hospital treatment, sometimes in intensive care, and can be life-threatening.
- **Psychological dissatisfaction** — Despite a technically successful result, a person may find it difficult to adjust to their new appearance. Discussing expectations openly before surgery reduces this risk.

General risks that apply to any surgical / medical procedure

- **Anaesthesia risks:** If general anaesthesia or sedation is used, its risks (nausea, sore throat, dental damage, allergic reaction and, very rarely, heart or lung problems) will be explained separately by the anaesthetist, and your consent will be obtained on a **separate anaesthesia consent form**.
- **Bleeding and blood transfusion:** The need for a blood transfusion in these procedures is very rare. In the event of serious bleeding, blood or blood products may be needed; these carry their own very rare risks (such as allergy or infection).
- **Infection:** A wound infection after surgery is usually treated with antibiotics; rarely, drainage (letting the pus out) may be needed.
- **Blood clots in the leg veins (DVT) and in the lungs (pulmonary embolism):** Very rare in short head and neck operations. Walking early reduces the risk. Hormone medicines, smoking, obesity and a previous history of clots increase the risk.
- **Drug allergy:** An allergic reaction may develop to the medicines, antiseptics, tapes or suture materials used.
- **Scarring and healing problems:** How scars form varies from person to person; very rarely a thick (hypertrophic) scar or keloid may form.
- **Risk to life:** Death in planned head and neck surgery is extremely rare today; however, no surgical procedure is entirely free of risk. A serious complication may require treatment in intensive care and may result in permanent disability.

This list does not include every possible complication; other unforeseeable and very rare events may occur. If a complication develops, your doctor will plan the necessary treatment and follow-up; this may require additional treatment, medication, procedures or a further operation.

9 When to contact us immediately

- **Nosebleed that does not stop or is very heavy** despite a few minutes of pressure and ice
- **Increasing nasal blockage, severe pain and swelling of the nose** in the days after surgery (may be a sign of a septal haematoma/abscess)
- **Fever above 38.5 °C**, increasing pain, redness or foul-smelling discharge from the nose
- **Sudden high fever, skin rash, dizziness or weakness** while packs are in place
- **Clear, watery discharge** from the nose, especially if it increases when you lean forward; or severe headache and neck stiffness
- **Blurred or double vision**, or rapidly increasing swelling and pain around the eye
- A blow to the nose or a sudden change in its shape
- Shortness of breath or chest pain

If any of these occur, call your doctor without delay: +90 288 502 50 20 · WhatsApp +90 555 333 21 20. If you cannot reach us, go to the nearest emergency department or call 112 (the emergency number in Türkiye and across Europe).

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10 Medication and everyday-life advice

- Do not blow your nose for the first 2 weeks; if you need to sneeze, sneeze with your mouth open.
- Clean the inside of your nose as shown to you and as often as recommended.
- For at least 6 weeks avoid knocks to the nose, glasses resting on the bridge of the nose and sleeping face down.
- Protect the skin of your nose from the sun for at least 3 months (high-factor sunscreen, a hat).
- If you have travelled from abroad, flying is usually safe after the splint has been removed (around day 7).
- Take the medicines prescribed for you (antibiotics, painkillers, etc.) at the recommended dose and for the recommended time. Do not take aspirin or similar medicines that may increase bleeding without asking your doctor.
- On flights or journeys longer than 4 hours after surgery, drink plenty of water, move around regularly and wear compression stockings if advised; this reduces the risk of leg clots.
- During recovery you are advised not to drink alcohol and to avoid smoking and nicotine products.
- Please attend all follow-up appointments. If you have travelled from abroad, follow-up after your return can continue by online consultation and photos/videos; if you need to see a local doctor, you will be given a written operation report and our recommendations.

11 Unexpected findings during the procedure, tissue samples and blood products

If an unforeseen situation arises during the procedure, your doctor will stay within the scope of the procedure you have consented to on this form. **Unless there is a risk to life or of serious harm to an organ**, no additional procedure that you have not consented to will be performed; in such a case the procedure will be brought to a close at a safe point, and you will be informed afterwards so that the decision can be made together with you. If your life is in danger, the emergency treatment needed to protect your life or the organ will be carried out.

Tissues removed during the procedure will be sent for **pathological examination** where necessary to confirm the diagnosis. Additional treatment or follow-up may be needed depending on the result.

Blood transfusion is very rarely needed in these procedures and will only be given when it is vital. If you do not accept blood or blood products (for example for religious reasons), you must tell us before the procedure; the possible consequences of this decision will be discussed with you separately and recorded under “Information specific to you”.

12 Photographs, image records and your personal data

Photographs/videos may be taken before and after the procedure for planning, follow-up and assessment of the result. These records form part of your medical file and are kept confidential. Your images will **never be used for advertising, promotion or social media.**

Your images may be used for any other purpose **only with your separate and explicit permission**. Declining will not affect your treatment in any way, and you may withdraw your permission at any time:

☐ I agree that my images are used **only for medical records and treatment follow-up**.

☐ I permit my images to be used for scientific publication and medical education **in a way that does not identify me** (face/eye area covered).

☐ I permit my images to be used for scientific publication and medical education **in a way that may identify me**.

☐ I **do not permit** scientific/educational use.

Your personal and health data are processed under the Turkish Personal Data Protection Law No. 6698 (KVKK) and the relevant health legislation, for the purposes of diagnosis, treatment and legal obligations, by persons bound by professional confidentiality. You may consult our Data Protection Information Notice for details.

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13 Information and notes specific to you (completed by the doctor)

Patient-specific additional risks (co-existing diseases, medicines, previous operations), details of the plan, the patient's expectations, questions asked and answers given. If there is nothing to add, write "none".

14 Patient declaration

- ☐ I have read and understood this form in my own language. I was given enough time to think; I had the opportunity to ask questions and received satisfactory answers.
- ☐ My condition, the aim of the recommended procedure, how it will be done, by whom and where, its approximate duration, expected benefits, likelihood of success, alternatives and what may happen if it is not done have been explained to me verbally and in writing.
- ☐ The common and potentially serious risks, and the possible consequences of these complications (including that further treatment or a repeat procedure/operation may be needed), have been explained to me. I understand that the outcome of any medical procedure **cannot be 100% guaranteed**.
- ☐ I have given accurate and complete information about my health, the medicines I take, my allergies and past illnesses. I understand that incomplete or incorrect information may increase the risks.
- ☐ I understand that following the instructions given before and after the procedure plays a part in the result, and that attending follow-up appointments is important.
- ☐ I know that I can withdraw my consent at any time before the procedure begins. I understand that signing this form does not remove the doctor's duty of care.
- ☐ I am making this decision of my own free will, without pressure from anyone.
- ☐ I understand that comparing my photographs from before and after surgery is part of assessing the result, and that computer simulations or example photographs are only meant to give an idea and do not guarantee the result.
- ☐ I understand that this procedure is **cosmetic**, that it is not medically necessary and that I always have the option not to have it. I have discussed my expectations with my doctor, and the realistic result that can be achieved has been explained to me.
- ☐ I understand that perfect symmetry or an exact copy of the appearance in a photograph cannot be guaranteed, that the final result depends on how my tissues heal, and that in some cases a correction procedure may be needed.

- ☐ **I GIVE MY CONSENT to the procedure described above.**
- ☐ **I DO NOT ACCEPT the procedure described above. The possible consequences of refusing have been explained to me.**

We recommend the patient writes in their own handwriting: "I have read and understood this form and I give my consent."

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15 Signatures

	FULL NAME	SIGNATURE	DATE / TIME
Patient			
Legal representative (parent / guardian) — relationship:			
Doctor providing information and performing the procedure			
Interpreter (if needed) — language:			
Witness (if needed)			

Interpreter's statement: I have translated this form and the doctor's verbal explanations to the patient / legal representative completely and accurately, in a language they understand.

FOR PATIENTS UNDER 18 OR WITH LIMITED DECISION-MAKING CAPACITY

For patients under 18, consent is given by a **parent or legal guardian**. The child is informed in a way appropriate to their age and maturity and their view is sought. For patients lacking capacity, consent is given by the legal representative.

☐ If we share parental responsibility, I declare that the other parent is aware of and agrees to this procedure.

☐ I declare that there is no court order or dispute concerning custody / guardianship (if there is, the relevant document has been shown to the doctor).

The child was informed in an age-appropriate way; the child's view: _____

This form has been prepared in two copies: one is kept in the patient's file and one is given to the patient / legal representative.

WITHDRAWAL OF CONSENT (COMPLETE ONLY IF YOU CHANGE YOUR MIND)

I withdraw the consent I gave above. The possible consequences of withdrawing have been explained to me.

Name, signature, date and time of patient/representative withdrawing consent: _____

This form has been prepared in accordance with the Turkish Patient Rights Regulation and the informed consent principles of the Turkish Medical Association. It complements, but does not replace, the verbal explanation given by your doctor.